



## Procedure

# Partnered Charting Pharmacist

## Scope

| Site         | Service/Department/Unit | Disciplines                 |
|--------------|-------------------------|-----------------------------|
| SCGH and OPH | Organisation Wide       | Medical, Nursing, Pharmacy, |

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## Introduction

This procedure outlines the process for Partnered Charting Pharmacists at Sir Charles Gairdner Osborne Park Health Care Group.

## Risk Statement

Non-compliance with this procedure will:

|                                       |                                     |                                   |                                     |
|---------------------------------------|-------------------------------------|-----------------------------------|-------------------------------------|
| Breach legislative requirements       | <input type="checkbox"/>            | Impact on Patient Quality of Care | <input checked="" type="checkbox"/> |
| Breach National/State/Hospital Policy | <input type="checkbox"/>            | Impact on Patient Safety          | <input checked="" type="checkbox"/> |
| Breach professional standards         | <input checked="" type="checkbox"/> | Misconduct                        | <input type="checkbox"/>            |
| Breach SCGOPHCG Mission & Values      | <input type="checkbox"/>            | Staff Safety                      | <input type="checkbox"/>            |
| Other:                                | <input type="checkbox"/>            |                                   |                                     |

## Procedure

Only Pharmacists that have been trained and deemed competent in partnered charting are able to complete the described activities.

### 1.0 Principles

Senior Pharmacists (AHPRA-registered) are authorised to undertake Partnered Charting within their scope of practice within services supported by medical staff members.

All medications charted by a credentialed pharmacist must be reviewed and signed by a medical staff member to be valid.

Pharmacists must have completed the approved training package and been deemed competent prior to undertaking partnered charting activities.

The clinical areas that can participate in partnered charting will be determined through consultation with the relevant Medical Heads of Department, Medical Co-Directors and the Chief Pharmacist.

The credentialed Clinical Pharmacist can perform these functions via two separate mechanisms:

1. On a referral basis
2. Embedded in a medical team

All charting must be written in purple pen and signed by the medical officer to be deemed valid.

All charting must occur in consultation with the medical officer and documentation in the patient's clinical record must meet the requirements of this procedure.

### 2.0 Scope of Clinical Practice

A summary of PCP Scope of Clinical Practice is outlined in Appendix 1.

#### 2.1 Within Scope

The following activities are within scope:

- All medications, unless stated in 2.2 **Out of Scope**. These include:
  - Fluid orders

- S4R medications
- Schedule 8 medications
- PRN medications
- STAT medication orders
- New medications under the direction of and in consultation with the treating team
- Regular medications that are appropriate to continue including amendments (eg dosage adjustment, withholding medicines, formulation conversion etc)
- Charting all medication formulations (eg oral, parenteral, inhaled, topical, sublingual, rectal etc)
- VTE prophylaxis
  - Pharmacist and treating team should discuss appropriateness together
  - the VTE risk assessment tool on the inside of the drug chart must be completed by the treating team or the Pharmacist before any charting
  - Chart on anticoagulation chart as appropriate
- Amendments to any medication orders following clinical pharmacist assessment, discussion with and direction of the treating medical team

## 2.2 Out of Scope

The following activities are out of scope:

- Charting any regular or new patient medications without a referral or request from the treating medical team
- Charting any regular patient medications without prior completion of a MHMP
- Charting certain specialised therapies
  - Clozapine (initiation or dose titration phases)
  - Specialised chemotherapy
    - Any therapy requiring Mosaiq prescribing or specialist input
    - Targeted parenteral therapies (eg nivolumab)
  - Live vaccines
    - BCG® vaccine
    - Imojev® Vaccine
    - M-M-R II® Vaccine
    - Priorix® Vaccine
    - Priorix-tetra Vaccine
    - ProQuad® Vaccine
    - Rotarix® Vaccine
    - RotaTeq® Vaccine
    - Stamaril® Vaccine
    - Varilrix® Vaccine
    - Varivax® Vaccine
    - Vivotif® Oral Vaccine
    - Zostavax® Vaccine
  - Intravenous insulin infusions
    - DKA, HHS
  - Specialised intravenous analgesia therapies
    - Epidural/regional infusions
  - Specialised ophthalmological therapies
    - Intravitreal / intracameral injections
  - Any other non-oral specialised therapies
    - Bladder/pleural instillations, intrathecal injections
- Charting or initiating any rectal preparations in a neutropenic or immunosuppressed patient

### 3.0 Identifying Suitable Patients

- Any potential patient for review will require a verbal referral or request from a member of the treating medical team. A credentialed clinical pharmacist may identify and recommend patients for referral by a medical officer.
- If that there are multiple requests for partnered charting, the PCP is expected to apply a system towards the prioritisation of all requests.
- The PCP should assess each case based on:
  1. Urgency (as dictated by the referring team)
  2. Patient-specific risk factors (if known)
  3. Medicine-specific risk factors (if available)
    - a. Must be assessed in the context of other factors discussed
  4. Reason for hospital admission (e.g. medication-related)

### 4.0 Patient Review

#### 4.1 Referral Based PCP

##### 4.1.1 Discussion with Medical Officer (MO)

- Prior to the review of the patient, the PCP and the MO must discuss the clinical plan for the referred patient.
  - Agreement must be made that the PCP will conduct partnered charting for this patient.
  - Agreement and clarification of the clinical plan proposed for the patient.
  - Any additional therapies recommended by the MO (eg antibiotics, analgesia, electrolyte supplementation etc) should be discussed and confirmed at this stage.

##### 4.1.2 Clinical Review

- The clinical review of the patient is generally one of the initial stages of the PCP process. All usual requirements for a pharmacy clinical review of a patient should be followed.

#### 4.1 Embedded in a Medical Team

- Pharmacist will clinically review the patient with the medical team via:
  - attendance on ward rounds
  - reviewing the patient together with the MO
- All usual requirements for a pharmacy clinical review of a patient should be followed.

### 5.0 Medication Reconciliation

- A Medication History and Management Plan must be completed prior to partnered charting of medications.
  - Refer to Pharmacy Standards, CLIN003 Medication Reconciliation: Completing the Medication History and Management Plan

### 6.0 Medication Charting

#### Patient Identification

1. Affix the patient addressograph/identification label to the designated sections on the WA HMC and all applicable charts for the patient (front and back of the medication

chart/s)

- Print the name of the patient underneath this label in the designated section 'First prescriber to print patient name and check label correct'

## Allergies and Adverse Drug Reactions (ADRs)

Record all known:

- Medication allergies with their corresponding reaction type
- Reaction date if available or known
- Moderate to severe adverse reactions to medications in the past (eg LFT dysfunction, severe nausea/vomiting, weight gain etc)
- Any medications or class of medications that should be avoided or used with caution in the patient
  - AVOID - Dopamine antagonists in Parkinson's Disease
  - CAUTION – Linezolid for a patient already receiving an SSRI

If there are no known drug allergies or ADRs for the patient, then the PCP should tick the 'Nil Known' box within the allergy section box.

If unknown, and unable to elucidate accurate details from the patient or available resources then tick the 'Unknown' box.

## General Housekeeping

- The year should also be completed on each medication chart in the relevant section.
- The ward should be documented on each medication chart in the relevant section
- If available, the weight and height of the patient should also be documented with the corresponding date of these measurements.

**Attach ADR Sticker**

**Allergies and adverse drug reactions (ADR)**  
☒ Nil known ☐ Unknown (tick appropriate box or complete details below)

| Medicine (or other) | Reaction / Type / Date | Initials |
|---------------------|------------------------|----------|
|                     |                        |          |
|                     |                        |          |
|                     |                        |          |
|                     |                        |          |
|                     |                        |          |
|                     |                        |          |

Sign: *[Signature]* Print: *A. Pharmacist* Date: *3/1/19*

**Regular Medicines**

**Patient Identification**  
 Affix patient identification label here and overleaf  
 URN: *123456*  
 Family name: *Amy Smith*  
 Given names: *DOB 01-01-45*  
 Address: *123456*  
 Date of birth: *01-01-45* Sex: M ☐ F ☐  
 Medicare No.: *123456* PBS/RPBS Entitlement No.: *123456*  
☐ Concessional or dependent RPBS or Safety Net Concession Card Holder ☐ Safety Net Entitlement Card Holder

First prescriber to print patient name and check label correct:  
*A. Smith*  
 Weight (kg):  Height (cm):  Date:  /  /   
 Year: *2019*

**Figure 1.** Example of the requirements for general NIMC charting (relating to patient details)

## Documentation of MHMP Completion

The PCP should indicate on the WA HMC that a MHMP has been completed.

- The PCP should tick the 'see WA MMP' box within the 'Medicines taken prior to presentation to hospital' section on the WA HMC (bottom half of the front page).

The PCP should also sign, print their name and include the date of MHMP completion in this section.

Other additional information that may be included (if available) include

- Details of the patient's regular community pharmacy



- Use of an administration aid such as a blister pack or dosette box.

**Medicines taken prior to presentation to hospital**  
(Prescribed, over the counter, complementary)

☒ See WA MMP    Own medicines brought in? Y ☐ N ☒ Administration aid (specify) WEBSTER PACK

| Medicine               | Dose and frequency | Duration | Medicine | Dose and frequency | Duration |
|------------------------|--------------------|----------|----------|--------------------|----------|
| Not for administration |                    |          |          |                    |          |

GP: \_\_\_\_\_ Community pharmacy: \_\_\_\_\_

Sign: \_\_\_\_\_ Print: \_\_\_\_\_ Date: \_\_\_\_\_ Medicines usually administered by: \_\_\_\_\_

**Figure 2.** Example of the documentation required to indicate the MHMP completion

## Medication Charting

Requirements relating to charting medications on the WA HMC (see figure 1)

1. The PCP must use a purple pen for all medication charting
2. Details that must be completed
  - a. Date of prescription
  - b. Printed medication name (full generic name unless exemption given)
 

Examples

    - i. ISMN is not acceptable; instead write 'isosorbide mononitrate'
    - ii. Insulins may be prescribed as their trade names
  - c. Medication form (if relevant)
  - d. Ticking the 'Slow Release' box if applicable
  - e. Route of administration
  - f. Dose and frequency
    - i. The use of trailing zeros is unacceptable (eg 2.0mg is incorrect; instead the correct prescription should include 2mg)
    - ii. Only approved medical abbreviations may be used for medication charting
  - g. Timing of dose/s, including the date within the nurse administration section
  - h. Indication for medication (if known)
3. The PCP should also annotate their name and contact number (pager, phone etc) on the medication chart to inform all other staff
4. All PCP documentation must be printed clearly and be legible
5. Each medication must be charted on their respective medication charts (eg insulin must be charted on the insulin medication chart only)
6. The S4R and S8 alerts should be circled when applicable
7. The PCP must sign each medication order
8. All relevant caution and advisory directions should be annotated by the PCP (where applicable)
9. Any pre-existing regular medication should be annotated with a ✓ alongside the order
10. Any new medications initiated during this admission should be annotated with new alongside the order
11. The PCP must ensure that each medication order is signed and printed by the treating medical officer (MO)

The image shows a sample NIMC medication order chart with handwritten entries and red annotations. The chart is divided into sections for two medications: Metoprolol and Digoxin. Annotations include:

- 2a**: Year 2019
- 2b**: Metoprolol
- 2e**: Digoxin
- 2f**: Prescriber signature
- 2h**: Indication
- 3**: Pharmacist signature
- 6**: Symbicort
- 7**: Imprint
- 8**: Rinse mouth
- 11**: Date and month
- 2g**: Time
- 2d**: Dose

**Figure 3.** Details that are required to be completed for each NIMC medication order to deem each prescription valid

### Other Medication Charts

The above requirements and principles should be applied to all other medication charts that the PCP may use for partnered charting

### Medication Order Amendments

- If there is an agreed plan by the treating MO/team to withhold any medications the PCP may annotate this on the medication chart using the (W) symbol within the nurse administration section of the medication chart.
- If any medication dosage adjustments are to occur, the amendments must be clearly documented on the medication chart alongside the medication order using the symbols ↑ or ↓ (to indicate either an increase or decrease in the medication dose from the regular dose).
- All order amendments must be made in consultation with the MO/Medical Team prior and these orders must be signed by the MO as per usual PCP processes.
- These same requirements apply to any changes the PCP suggests in regards to the formulation or route of administration of the patient's regular medications.
  - Eg conversion of the patient's oral dose to a therapeutically bioequivalent parenteral dose or substitution of the oral tablet dose for an oral liquid formulation

## Medication Reconciliation

The PCP must complete a medication reconciliation following the partnered charting process.

- This involves reconciliation of the medication chart against the completed MHMP.
- There should also be assessment and/or review of each medication charted against the clinical context of the patient and their current hospital admission to determine appropriateness of continued therapy

## 7.0 Post Charting Responsibilities

### Follow-up with Treating Medical Team

- PCP must discuss each case with the medical team following full review to highlight
  - Clinical summary of the case
  - Any concerns from a pharmacist perspective
  - Any PCP recommendations, amendments or changes to the patient's medications
- All partner charted medication charts must be compiled and presented to the medical team together to ensure all relevant documents are received
- PCP must ensure that each individual medication order is signed by the treating MO to validate each prescription order.
- If not able to be signed, this legislative responsibility must be handed over to the treating medical team and the responsibility for completion of this requirement will be transferred to the treating medical team.

### Follow-up with Nursing Staff

- Following review and signing of each order by the MO, the medication charts should be presented to the nursing staff to
  - Inform the nursing staff of the charted medications and/or changes
  - Inform the nursing staff of new medications the patient has been commenced on
  - Discuss with nursing staff which medications will need to be ordered from the Pharmacy Department (eg non-imprest or restricted therapies)
    - If Patient's Own Medications (POMs) are available then these medications may be a suitable option for use during the hospital admission if provided consent by the patient/family/carer)
- If the medication order/s are unable to be signed by the medical staff at this stage, this must be relayed to the nursing staff
  - The nursing staff must be informed that the medication order/s are not valid until they are individually signed by an authorised prescriber
  - The medication must not be administered to the patient until each medication order is signed by an authorised prescriber

### Medication Ordering

- The PCP should order any medications that are unavailable on the ward following discussion with the nursing staff.

### Documentation Requirements

The PCP is expected to document all relevant information relating to each case on the appropriate hospital stationary

- Best possible medication history



- WA Medication History and Management Plan document (MR 410/804)
- Medications on appropriate medication charts / forms

### 7.1 On a referral basis

- Written entry of details of the case (into inpatient notes MR 710)
  - Patient reviewed
  - Referring MO or team (for pharmacist partnered charting)
  - Date and time of entry
  - Pharmacist name (printed)
  - Details of any relevant discussions, decisions or agreed plans between the PCP and either MO or nursing staff using approved ISOBAR format
    - Identify – details of the patient reviewed (name, gender, age,)
    - Situation - brief description of reason for hospital admission
    - Observation – include only if relevant to the situation/review
    - Background - relevant background medical information pertaining to the patient
    - Assessment/Agreed plan - document the plan that was discussed and agreed upon with the referring MO or medical team. Also note any additional findings or concerns that have arisen following PCP clinical review
    - Recommendations - document any recommendations or suggestions in regards to this patient following clinical review
  - Document any other relevant details that will be beneficial to other staff members (eg POMs, Webster pack, CPOP, non-compliance to medications etc)
  - Record any handover provided to other pharmacists or nurses
  - Signature, contact details and designation of the Pharmacist submitting medical notes entry

### 7.2 Embedded in a medical team

- Pharmacist presence must be documented as part of the medical team entry (into inpatient notes MR 710)
  - All medication changes are to be documented as part of the medical team entry
  - The clinical pharmacist will counter sign entry when medication changes have occurred on medication chart
    - The pharmacist must print their name
    - Pharmacist must document date

### Handover to Pharmacist Colleague/s

- Any relevant information pertaining to the patient and the PCP process should be communicated to the relevant pharmacy colleagues (eg pharmacist on receiving ward, late shift pharmacist, on call pharmacist etc).
- Acceptable methods of handover (in order of preference)
  - Verbal handover (in person, via phone)
  - Written handover (via email)
  - Electronic handover (via Microsoft Teams)

## Compliance, monitoring and evaluation

An audit schedule will be introduced to reinforce periodic independent assessment of the PCP training program.

The purpose of this schedule will be to provide an objective examination of the processes and outcomes of the PCP program.

Results from the auditing process will enable quality improvement initiatives to be identified and implemented to advance the overall PCP program.

| Monitoring                            | DETAILS                                                                                                                                              | FREQUENCY  |
|---------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------|------------|
| <b>AUDIT OF CREDENTIALING PROCESS</b> | Ensure all Pharmacists performing PCP activities have been credentialed as per the Partnered Charting Pharmacist Credentialing Process.              | 12 monthly |
| <b>REVIEW OF CLINICAL PROCESS</b>     | 2 MiniCEX's performed to review process.<br>Feedback sought from medical officers working with the Partnered Charting Pharmacist to review feedback. | 12 monthly |

## Related Documents

- [Western Australian Medicines and Poisons Act 2014](#)
- [Western Australian Medicines and Poisons Regulations 2016](#)
- [WA Department of Health MP 0077/18 Statewide Medicines Formulary Policy](#)
- [WA Department of Health MP 0078/18 Medication Chart Policy](#)
- [WA Department of Health MP 0131/20 High Risk Medication Policy](#)
- [WA Department of Health MP 0122/19 Clinical Incident Management Policy](#)
- SCGOPHCG Medicines Management Policy
- SCGOPHCG High Risk Medications Policy
- SCGOPHCG Smart Infusion Pumps and Dose Error Reduction Software Policy
- NMHS Medicines Management

## References

- 1) Tong EY, Roman CP, Smit DP, Newnham H, Galbraith K, Dooley MJ. Partnered Medication Review and Charting Between Pharmacist and Medical Officer in the Emergency Short Stay and General Medicine Unit. AENJ. 2015; **18**: 149-155
- 2) Tong EY, Roman CP, Mitra B, Yip G, Gibbs H, Newnham H, Smit DP, Galbraith K, Dooley MJ. Partnered Pharmacist Charting on Admission in the General Medical and Emergency Short-stay Unit – a Cluster-Randomised Controlled Trial in Patients with Complex Medication Regimens. Journal of Clinical Pharmacy and Therapeutics. 2016; 1-5
- 3) Tong EY, Mitra B, Yip G, Galbraith K, Dooley MJ. Multi-site evaluation of partnered pharmacist medication charting and in-hospital length of stay. British Journal of Clinical Pharmacology. 2020; 86(2):285-290
- 4) Hitchen S, Sinclair V, McLennan C, Kim B, Huynh C, Tran S, Rawlins M. Safer

Prescribing with Partnered Pharmacist Charting in the Acute Medical Setting. Fiona Stanley-Fremantle Hospitals Group. QI GEKO Project #26656. Poster Presentation. Annual IMPROVE Conference.

- 5) SHPA Fact Sheet: Risk Factors for Medication-related Problems. The Society of Hospital Pharmacists of Australia. <http://www.shpa.org.au> (accessed April 2020)
- 6) Clinical Excellence Commission (CEC). High-Risk Medicines A PINCH. <http://www.cec.health.nsw.gov.au/patient-safety-programs/medication-safety/high-risk-medicines/A-PINCH> (accessed April 2020)

## Authorisation & Version Control

|                      |                                                                                                                  |                |        |                         |
|----------------------|------------------------------------------------------------------------------------------------------------------|----------------|--------|-------------------------|
| Policy Sponsor       | Area Director Clinical Services                                                                                  |                |        |                         |
| Policy Contact       | Deputy Chief Pharmacist (Clinical)                                                                               |                |        |                         |
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| Endorsed by:         | SCGOPHCG Policy and Guideline Committee                                                                          |                |        | Date: 12/07/2023        |
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| Standards Applicable | NSQHS Standards (Version 2):  |                |        |                         |

For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

This document can be made available in alternative formats on request for a person with a disability.

## APPENDIX 1 – PCP Scope of Clinical Practice

| WITHIN SCOPE                                                                                                                                                                                                                                                                                                                                              | OUTSIDE OF SCOPE                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                |
|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| All medications, excluding those stated in 'outside of scope section'                                                                                                                                                                                                                                                                                     | Charting any regular or new patient medications without a referral or request from the treating medical team                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    |
| Charting all medication formulations (eg oral, parenteral, inhaled, topical, sublingual, rectal etc)                                                                                                                                                                                                                                                      | Charting any regular patient medications without prior completion of a MHMP                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     |
| Regular medications that are appropriate to continue including amendments (eg dosage adjustment, withholding medicines, formulation conversion etc)                                                                                                                                                                                                       | Charting certain specialised therapies <ul style="list-style-type: none"> <li>• Clozapine (initiation or dose titration phases)</li> <li>• Specialised chemotherapy               <ul style="list-style-type: none"> <li>○ Any therapy requiring Mosaik prescribing or specialist input</li> <li>○ Targeted parenteral therapies (eg nivolumab)</li> </ul> </li> <li>• Live vaccines (see below)</li> <li>• Intravenous insulin infusions               <ul style="list-style-type: none"> <li>○ DKA, HHS</li> </ul> </li> <li>• Specialised intravenous analgesia therapies               <ul style="list-style-type: none"> <li>○ Epidural/regional infusions</li> </ul> </li> <li>• Specialised ophthalmological therapies               <ul style="list-style-type: none"> <li>○ Intravitreal / intracameral injections</li> </ul> </li> <li>• Any other non-oral specialised therapies               <ul style="list-style-type: none"> <li>○ Bladder/pleural instillations, intrathecal injections</li> </ul> </li> </ul> |
| S4R medications                                                                                                                                                                                                                                                                                                                                           |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 |
| Schedule 8 medications                                                                                                                                                                                                                                                                                                                                    |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 |
| PRN medications                                                                                                                                                                                                                                                                                                                                           |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 |
| STAT medication orders                                                                                                                                                                                                                                                                                                                                    |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 |
| VTE prophylaxis <ol style="list-style-type: none"> <li>1. Pharmacist and treating team should discuss appropriateness together</li> <li>2. the VTE risk assessment tool on the inside of the drug chart must be completed by the treating team or the Pharmacist before any charting</li> <li>3. Chart on anticoagulation chart as appropriate</li> </ol> |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 |
| Fluid orders                                                                                                                                                                                                                                                                                                                                              |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 |
| Amendments to any medication orders following clinical pharmacist assessment, discussion with and direction of the treating team                                                                                                                                                                                                                          |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 |
| New medications under the direction of and in consultation with the treating team                                                                                                                                                                                                                                                                         |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 |
|                                                                                                                                                                                                                                                                                                                                                           | Charting or initiating any rectal preparations in a neutropenic or immunosuppressed patient                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     |

**APPENDIX 2 – List of live vaccines that are out of scope of the PCP**

| <b>Live Vaccines</b>              |                                                   |
|-----------------------------------|---------------------------------------------------|
| BCG <sup>®</sup> vaccine          | bacille Calmette-Guérin, live                     |
| Imojev <sup>®</sup> vaccine       | Japanese encephalitis, live attenuated            |
| M-M-R II <sup>®</sup> vaccine     | Measles- mumps-rubella, live attenuated           |
| Priorix <sup>®</sup> vaccine      | Measles- mumps-rubella, live attenuated           |
| Priorix-tetra vaccine             | Measles- mumps-rubella-varicella, live attenuated |
| ProQuad <sup>®</sup> vaccine      | Measles- mumps-rubella-varicella, live attenuated |
| Rotarix <sup>®</sup> vaccine      | Human rotavirus, live attenuated                  |
| RotaTeq <sup>®</sup> vaccine      | Human-bovine rotavirus, live attenuated           |
| Stamaril <sup>®</sup> vaccine     | Yellow fever, live attenuated                     |
| Varilrix <sup>®</sup> vaccine     | Varicella-zoster, live attenuated                 |
| Varivax <sup>®</sup> vaccine      | Varicella-zoster, live attenuated                 |
| Vivotif <sup>®</sup> oral vaccine | Typhoid, live attenuated                          |
| Zostavax <sup>®</sup> vaccine     | Herpes-zoster, live attenuated                    |